Obstetric medicine requires careful assessment of maternal and fetal physiology throughout all gestational ages. In particular hypertensive disorders of pregnancy such as preeclampsia and gestational hypertension represent leading causes of maternal morbidity worldwide. Diagnostic criteria historically relied upon proteinuria alongside sustained blood pressure elevations however modern international guidelines emphasize end organ dysfunction including renal impairment hepatic involvement neurological complications and hematological abnormalities. Management strategies necessitate rigorous blood pressure control using first line agents such as labetalol nifedipine and hydralazine with magnesium sulfate administered for seizure prophylaxis in severe disease. Timely delivery remains the definitive intervention balanced against neonatal risks of prematurity. Postpartum surveillance is equally crucial as hemodynamic shifts and vascular resistance fluctuations frequently persist for several weeks following parturition requiring extended outpatient follow up and cardiovascular risk counseling for subsequent life stages. Longitudinal cohort studies demonstrate elevated risks of chronic hypertension coronary artery disease and stroke in women with previous preeclampsia highlighting the need for structured multidisciplinary transition into preventive primary care cardiology programs. Furthermore comprehensive patient education regarding cardinal warning symptoms including persistent visual disturbances epigastric pain and refractory headaches facilitates early triage and reduces catastrophic outcomes. Implementing standardized institutional emergency protocols ensures consistent clinical management across diverse obstetric units. Continuous professional development programs for obstetric nursing and medical personnel emphasize the critical importance of rapid risk recognition and adherence to clinical protocols. Multidisciplinary clinical collaboration between maternal fetal medicine specialists obstetric anesthesiologists and neonatal intensive care teams substantially improves maternal and perinatal outcomes in complex tertiary healthcare settings worldwide.